

Lecture note on WCS82: Painful Red joint

Carmen Ho

Acute monoarthritis is treated as semi-emergency problem in medicine. The first and foremost, acute monoarthritis should be considered is septic arthritis until proven otherwise.

An accurate diagnosis of monoarthritis depends on good history taking, physical examination that are supplemented by the appropriate investigations. Out of all the investigations, joint aspiration with synovial fluid analysis for leucocyte, Gram's stain, culture, and crystal, is the most important test.

The common causes of monoarthritis include septic arthritis, crystal deposition arthritis e.g gouty arthritis and pseudogout, haemarthrosis. Do include other polyarticular diseases with monoarticular presentation. Examples include rheumatoid arthritis, psoriatic arthritis and spondyloarthritis.

Key learning points

- Bacterial septic arthritis should be treated promptly with antibiotics according to culture sensitivity.
- Surgical intervention is indicated in some patients so always get the orthopaedics surgeon informed.
- Acute crystal deposition arthritis is treated symptomatically with NSAIDs, corticosteroids or colchicine.
- Remember to implement long term prevention treatment in patients with recurrent attacks of gouty arthritis.
- The definitive diagnosis of haemarthrosis is joint aspiration. Treatment is by treating the "treatable" underlying cause.